

Slide 5 Reading Notes

Recommendations | Meds Consulting LLC

Opening

This slide turns the analysis into recommendations for NHN. The recommendations cover short-term actions, long-term strategy, estimated cost, expected benefits, implementation timeline, and potential risks.

The top row summarizes the expected case. The implementation cost assumption is 1.75 million dollars. The expected reduction avoids about 518 readmissions, generates about 8.81 million dollars in net savings, and produces an estimated 5.03 times ROI.

1. Short-term actions

In the first 0 to 3 months, NHN should pilot pre-discharge readmission risk scoring for emergency admissions and skilled nursing discharges. These groups show elevated readmission risk, so they are a practical starting point for targeted discharge review.

NHN should also close follow-up documentation and completion gaps. Unknown follow-up status has one of the highest observed readmission rates, and 2,587 records lack follow-up status. Fixing this workflow improves both patient tracking and data quality.

The third short-term action is to approve dashboard KPIs and readmission reduction targets. This creates a shared definition of success before the pilot expands.

2. Long-term strategy

The longer-term strategy is to expand care coordination for patients with high chronic disease burden and repeat admissions. These groups have materially higher readmission rates, so targeted support should create more value than spreading resources evenly.

NHN should also track ROI, penalty exposure, intervention performance, and model performance monthly. This keeps the program connected to clinical outcomes, operations, and finance.

3. Implementation timeline

The implementation timeline has three phases. In 0 to 3 months, NHN pilots risk scoring, fixes the follow-up workflow, and aligns KPI governance. In 3 to 6 months, NHN expands targeted care coordination and publishes the executive dashboard. In 6 to 12 months, leadership reviews ROI and refreshes the model using new outcomes and operational feedback.

Phase	Focus
0-3 months	Risk-score pilot, follow-up workflow fix, KPI governance
3-6 months	Targeted care coordination expansion and executive dashboard
6-12 months	Monthly ROI tracking and model refresh

4. Costs and expected benefits

The expected scenario assumes a 10 percent readmission reduction and a 1.75 million dollar implementation cost. Based on the ROI model, this would avoid about 518 readmissions and generate about 8.81 million dollars in net savings.

These numbers should be treated as a planning estimate. Finance should validate cost, penalty, and intervention assumptions before NHN treats the scenario as a budget commitment.

5. Potential risks

There are three main risks. First, risk score adoption may stall if clinical workflow ownership is unclear. Second, follow-up documentation gaps may continue to hide operational performance issues. Third, ROI estimates depend on validated cost, penalty, and intervention assumptions.

The decision ask is to approve a 0 to 3 month pilot focused on risk scoring, follow-up visibility, and discharge review. This gives NHN a controlled way to test the recommendation before scaling it across the system.